F A X S H E E T

Date:	03/02/2026 09:08:46 AM
To:	PAM Health at home
Subject:	Patient Document
Fax Number:	(512)295-7070
To Company:	
From Name:	Martinez,Sara
From Company:	HTMG LAS PALMAS2
From Facility:	HTMG LAS PALMAS2
Support Contact:	210-436-7402
Number of Page(s):	12

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Fax

To: CRUZ, MAX	**From:** Pam Health at Home - Texas
Fax: 12104367398	**Pages:** 11
Phone:	**Date:** 02-20-2026 4:55 PM
Re: Order(s) for Signature	

Comments: URGENT

Please review, sign, date and return asap, today if possible.

Claudia M. Orders Team
ph 210-728-9736
fax 512-295-7070

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

IMPORTANT - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error please contact the sender and destroy this facsimile.

Thank you!

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Ph: 512-295-7000; Fax: 512-295-7070

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Order No:485-22524

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
98416984701	12/09/2025	12/09/2025 - 02/06/2026	8594	262009456

6. Patient's Name and Address
Zamarron, Norberto I (8594)
115 Darson Marie, #104
San Antonio, TX 78226
Phone: (726) 247-7735

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 06/06/1952 **9. Sex:** ☒ M ☐ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Bayer Low Dose Aspirin 81 mg tablet,delayed release [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Coreg 3.125 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	2 times a day	oral	LS
Docusate sodium 100 mg capsule [Amount Of/ Dosage Form: 0 Capsule]	1	at bedtime	oral	LS
Humalog U-100 Insulin 100 unit/mL subcutaneous solution [Amount Of/ Dosage Form: 0 Milliliter]	s/s	3 times a day	subcutaneous	LS
Hydrochlorothiazide 12.5 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	every other day	oral	LS
Ketoconazole 2 % shampoo [Amount Of/ Dosage Form: 0 shampoo]	1 application	daily	topical	LS - PRN/As Needed
Levothyroxine 100 mcg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Midodrine 10 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	3 times a day	oral	LS
Motion Sickness (meclizine) 25 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	3 times a day	oral	LS - PRN/As Needed
Plavix 75 mg tablet [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Protonix 40 mg tablet,delayed release [Amount Of/ Dosage Form: 0 Tablet]	1	daily	oral	LS
Scopolamine 1 mg over 3 days transdermal patch [Amount Of/ Dosage Form: 0 patch 3 day]	1	every 3 days	transdermal	LS
Tresiba U-100 Insulin 100 unit/mL subcutaneous solution [Amount Of/ Dosage Form: 0 Milliliter]	20 units	at bedtime	subcutaneous	LS

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
I12.9	Hypertensive chronic kidney disease with stage 1 through stage 4 chronic kidney disease, or unspecified chronic kidney disease	12/09/2025	E

12. ICD-9 / 10 CM **Surgical Procedure**

Code	Code Description	Date	O/E

Medicare#:

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13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
E11.22	Type 2 diabetes mellitus with diabetic chronic kidney disease		
N18.30	Chronic kidney disease, stage 3 unspecified		
I26.99	Other pulmonary embolism without acute cor pulmonale		
S06.0X9D	Concussion with loss of consciousness of unspecified duration, subsequent encounter		
I25.10	Atherosclerotic heart disease of native coronary artery without angina pectoris		
I70.0	Atherosclerosis of aorta		
E78.2	Mixed hyperlipidemia		
R29.6	Repeated falls		
R42	Dizziness and giddiness		
R63.4	Abnormal weight loss		
Z91.81	History of falling		
Z79.4	Long term (current) use of insulin		
Z79.02	Long term (current) use of antithrombotics/antiplatelets		
Z79.82	Long term (current) use of aspirin		
Z90.5	Acquired absence of kidney		
Z85.528	Personal history of other malignant neoplasm of kidney		

14. DME and Supplies:
DME: No DME/Supplies required

15. Safety Measures:
911 Emergency / Protocol, Anticoagulant Precautions, DIABETIC PRECAUTIONS, Emergency Plan Developed, Fall / Safety Precautions, Fall Precautions, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
No Concentrated Sweets,
HH

17. Allergies:

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Medication Allergies: Penicillins, Sulfa (Sulfonamide Antibiotics)

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☐ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☑ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☐ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☑ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:

Frequency
SN Frequency: 2W1, 1W8 Starting Week of 12/09/2025, SN PRN Visits/Hours: 3 (Falls, BP, S/S infections),PT Frequency: 1W1, 2W1, 1W3 Starting Week of 12/09/2025,

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 300
Weight: < 225 or > 238
Pain: > 7

SN
ANTICOAGULANT THERAPY:
- SN to instruct patient/caregiver on medication side effects to report to SN or Physician.

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BEST PRACTICE FALL PREVENTION:

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

GENERAL: BEST PRACTICE DIABETES:

- Clinician to assess/perform monitoring for the presence of skin lesions on the lower extremities and perform patient/caregiver education on diabetic foot care.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - CAD:

- SN to instruct patient/caregiver on a Heart Healthy diet

CARDIOVASCULAR - HTN:

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.

PT

PHYSICAL THERAPY:

- Evaluation and Treatment
- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: Balance, coordination, and/or agility training
- Therapeutic Exercises: Strength training

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- Gait Training: Even and uneven surfaces and/or natural terrain
- Transfer Training : Supine <>Sit, Sit <> Stand, Bed <> Chair, On/Off toilet or commode, In/Out shower or tub , Floor , car
- Bed Mobility: Scoot/roll/bridge

Assessment Summary

FOCUS OF CARE: HTN uncontrolled, dizziness, PE, frequent Falls with anticoagulation therapy, medication education and compliance reinforcement cardiovascular comorbidities
Dwelling: patient lives on the bottom floor of a assisted living complex patient lives alone patient has no pets patient requires moderate assistance for ADLs at this time
SN Admitted 73 yo male , under Dr. Max Cruz, for strengthening and conditioning secondary to multiple falls and deconditioning patient was admitted on about November 26 to the hospital for portable Falls for a couple of days patient was transferred to rehab and discharged home on December 5 while in the hospital patient's medication's were changed and appears to be related to polypharmacy complications and blood pressure lability , patient was previously on hospice that was taken off by son and patient , patient has multiple cardiovascular morbidities and diagnosed SA over patient did not use any type of supplemental oxygen or sleep apnea machines at this time, patient is diabetic and is monitoring blood sugars, patient was seen for blood draws on Monday by new PCP and we'll see patient on Monday for first visit with new PCP, patient with a medical history of a PE coronary artery disease multiple falls chronic kidney disease stage three diabetes type two with insulin hypertension USA hypothyroidism and renal cancer that was in submission in 2023 after a right partial nephrectomy is the only surgery listed by the patient , patient would benefit from skilled nursing for education and ongoing reinforcement of medication compliance and comorbidity monitoring and compliance as well as physical therapy for strengthening conditioning patient remains a high risk at this time .Patient was identified with 2 forms of ID: insurance card and picture ID. .Patient agreed w/ provision of HHC. Consents signed for care, Pt/Cg instructed on Patient Rights and responsibilities, HIPAA privacy act, purpose of Advance Directives, complaint procedure with Texas Health and Human Services Commission and Consumer Rights and Services Division DADS hotline number provided, Administrators name and number for any complaints with agency staff, Clinical Manager name and phone number in home folder, agency drug policy, agency transfer and discharge policies, Emergency Preparedness plan and availability of PPE for patient/caregiver. Written materials reviewed with patient and left in the home chart. Patient and caregiver involved in plan of care creation and is/are in agreement with the plan of care. Pt/cg instructed PAM home health . RN available 24 hours per day/7 days per week. Pt/Cg verbalized understanding contacting on-call nurse or PAM home health office at # 1-866-611-1618 or after hours 512-212-0067. Past medical history: see above. SN performed whole body assessment and VS taken, , VS WNL for patient, patient is alert and oriented and greeted sitting in the living room with the sun present patient denies any shortness of breath while at rest however her does experience dyspnea with exertion patient denies any chest pain no edema patient has multiple cardiovascular comorbidities however no acute complications or issues at this time, patient is consonant a bowel and bladder and denies UTI or bowel complications bowel movements every other day bowel movement yesterday and today, patient does report decreased appetite however with the poly pharmacy changes patient states appetite slowly increasing patient has had significant weight loss over last few months, patient has no wounds or breakdowns, patient utilizes rolling walker and wheelchair for mobility at this time patient remains a moderate assist at this time for ADLs patient denies any pain , patient handbook was reviewed with patient emphasis on patient medication compliance medication set up was performed old medication's were removed similar medication's or duplicate medications were combined patients medications from the pharmacy were retrieved and patient' s PCP was called to notify for admission and for review of all medications upon follow up next week as patient had history of polypharmacy and most likely leading complication for patients multiple falls with orthostatic hypertension tested patient while present patient did experience smiled orthostatic hypertension related symptoms discussed taking position changes slowly rising slowly taking time for position changes and ambulating as well as monitoring blood pressure pressures before all administrations of medication's with parameters to hold medication's if blood pressure is too high or too low as well as monitoring patient's weight daily and

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monitoring patient's blood sugars and monitoring all activity analogue patient verbalized compliance will require ongoing education reinforcement . SN completed medication reconciliation and instructed patient on new and/or current medication list: Patient/CG was able to verbalize actions, side effects, and correct schedule/dosages of his/her meds. POC reviewed and agreed upon by patient/patient's family/CG and clinician. Physician has been notified of SOC and agrees to the POC. Patient left safe and stable in home
Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive devices and the assistance of another person in order to leave home. It requires considerable effort that is very taxing on the patient to leave home AEB recent hospitalization for multiple and frequent Falls patient was previously on hospice is no longer on hospice and now is dealing with ongoing cardiac comorbidities including the multiple files secondary to orthostatic hypotension and polypharmacy with a history of hypertension diabetes dizziness and vertigo requiring use of a walker and sometimes use of a wheelchair or requiring taxing effort to leave home.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. Zamarron, Norberto I is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 12/05/25 by Nnabugo D Ozurumba MD and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 12/09/25.

Risk of Hospitalization

Risks: History of falls (2 or more falls - or any fall with an injury - in the past 12 months); Unintentional weight loss of a total of 10 pounds or more in past 12 months; Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Currently taking 5 or more medications;DM and falls

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Interventions: Referrals - SN,PT; Patient/Family Education; Front-loading Visits; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); More than 2 Secondary Diagnoses (M1022 & M1024); Confusion (M1710); Lives Alone (M1100) ; Inadequate Support Network (M1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

Primary Language -english;
Sleep Rest:Adequate;
Other:patient is calm cooperative and engaged, patient denies HI and SI or active depression.

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

POA; Copies Requested
Comments -: full code , son is POA
Patient's Advance Directive - Appointment of Health care agent;
Have an Advanced Care Plan and it is documented in the medical record: Yes - Medical Treatment Preferences;
Have a Surrogate Decision Maker and information documented in the medical record: Yes;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Severe SOB, SOB upon exertion;
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive
devices and the assistance of another person in order to leave home. It requires considerable effort that is very taxing on the patient to leave home AEB recent hospitalization for multiple and frequent Falls patient was previously on hospice is no longer on hospice and now is dealing with ongoing cardiac comorbidities including the multiple files secondary to orthostatic hypotension and polypharmacy with a history of hypertension diabetes dizziness and vertigo requiring use of a walker and sometimes use of a wheelchair or requiring taxing effort to leave home

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

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Goals

SN

ANTICOAGULANT THERAPY: - LTG
- LTG-Patient will be free of adverse s/s of anticoagulant therapy at end of episode.

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

GENERAL: BEST PRACTICE DIABETES: - STG
- STG-Patient/Caregiver will demonstrate ability to perform proper diabetic foot care as evidenced by Keeping a daily log by 6 weeks.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - CAD: - LTG
- Patient/Caregiver will be able to verbalize ways to manage CAD and will be free of complications through the certification period.

CARDIOVASCULAR - HTN: - LTG
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

PT

FUNCTIONAL MOBILITY: - STG
- STG-Pt will demonstrate improved functional mobility with getting in/out of bed mobility and sit to stand transfers with independence and decrease caregiver burden.

MOBILITY: - LTG
- LTG-Pt will demonstrate an increase in strength via the TUG with an improved score of 13 seconds to decrease risk for falls.

MOBILITY/GAIT: - LTG
- LTG-Patient will ambulate on level surface a continuous distance of 300ft using 2 WW with SBA to improve safety at home and enhance ability to walk to bathroom from living room or bedroom.

STRENGTH: - LTG
- LTG-Patients will be independent with BUE/BLE home exercise programs in order to maintain gains made in-home therapy.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 12/09/2025
Signed By: Rogers, Jesse - RN on 12/09/2025 12:36 PM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

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24. Physician's Name and Address
MAX CRUZ MD
911 Castroville Rd.
San Antonio, TX 78237,NPI: 1235713736, License#: U9882,Phone: (210) 436-7402, Fax: (210) 436-7398

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:

3/2/2026

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.

Medicare#:

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